SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

FOLFIRINOX followed by chemoradiotherapy · rectal cancer

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Locally advanced rectal cancer · NCCP 00691 · version 4 · 14-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This is a planned course before surgery assessment for locally advanced rectal cancer. Six FOLFIRINOX cycles are followed by capecitabine with pelvic radiotherapy. The team reviews response and the next part of your overall rectal-cancer plan.

Schedule
Phase 1: six FOLFIRINOX cycles, one every 14 days, each with a 46-hour fluorouracil pump. Phase 2: capecitabine twice daily on Days 1–5 of each seven-day week for five weeks, with radiotherapy. Your team supplies the separate radiation calendar.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ DPD result and severe fluoropyrimidine risks reviewed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Oxaliplatin
PLATINUM CHEMOTHERAPY

- ☐ Cold-triggered tingling / throat discomfort
- ☐ Persistent sensory neuropathy, sometimes lasting after treatment
- ☐ Allergic / infusion reaction
- ☐ Low blood counts and infection
- ☐ Nausea, diarrhoea and fatigue
- ☐ Rare serious lung or blood complications

Folinic acid
CHEMOTHERAPY MODULATOR

- ☐ Enhances fluorouracil activity and toxicity
- ☐ Rare allergic reaction
- ☐ Diarrhoea / mouth soreness in combination

Irinotecan
TOPOISOMERASE I CHEMOTHERAPY

- ☐ Early cramps, sweating and diarrhoea
- ☐ Delayed diarrhoea and dehydration
- ☐ Low blood counts / serious infection
- ☐ Nausea, mouth soreness and fatigue
- ☐ Hair thinning or loss
- ☐ Rare serious lung or allergic reactions

Fluorouracil
CHEMOTHERAPY

- ☐ Diarrhoea, dehydration and mouth ulcers
- ☐ Low blood counts / infection
- ☐ Hand-foot skin soreness
- ☐ Cardiotoxicity including chest pain / ischaemia
- ☐ Severe early toxicity including DPD-related toxicity
- ☐ Pump / central-line complications

Capecitabine
ORAL CHEMOTHERAPY

- ☐ Diarrhoea and dehydration
- ☐ Painful hands / feet
- ☐ Mouth soreness and reduced intake
- ☐ Low blood counts / serious infection
- ☐ Chest pain / heart problems
- ☐ Severe early toxicity, including DPD-related toxicity

Pelvic radiotherapy
LOCAL RADIATION TREATMENT

- ☐ Bowel irritation / diarrhoea
- ☐ Pelvic skin soreness
- ☐ Urinary symptoms
- ☐ Fatigue
- ☐ Possible lasting bowel / bladder effects
- ☐ Sexual function / fertility effects

Confirm the individual aim, likely benefit and alternatives, DPD result, the 46-hour pump, disconnection and urgent contacts. Explain early irinotecan symptoms separately from delayed diarrhoea. Explain cold sensitivity and potentially persistent neuropathy. Agree the two phases, radiation planning, tablet instructions, fertility and potential bowel, bladder and sexual effects.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00691 v4.

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